Navigating New Disability as a Sociotechnical Process

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Abstract

Adults experiencing new disabilities must often decipher symptoms, seek diagnosis, develop treatment plans with clinicians, and build everyday adaptations, all within constrained healthcare systems. This paper examines how recently disabled adults in the United States navigate this process and use technology, and other resources, to do so. From interviews with 14 adults, we contribute three insights for accessibility research. First, participants pieced together digital tools and online resources to coordinate care and build credibility with physicians across fragmented medical systems. Second, technology use went beyond accessing information, to managing trust, legitimacy, and communication within patient-clinician relationships. Third, disability identity, support needs, and technology use did not emerge in lockstep: participants often sought diagnosis-specific tools, accommodations, and support while remaining uncertain about disability as a category and grappling with permanence and grief. We discuss implications for designing technologies that support recent disability while sensitive to fluctuating identities, needs, and care pathways.

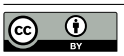
CCS Concepts

• **Human-centered computing → Empirical studies in accessibility**.

Keywords

accessibility, new disability, healthcare tools, clinical tensions, patient-clinician relationships, patient research practices, sociotechnical

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1 Introduction

The journey from first symptoms to diagnosis, treatment, and everyday adaptation can be especially complex for adults who navigating a new disability later in life. Alongside physical, cognitive, or emotional changes, people must quickly learn how to seek care within complex healthcare and insurance systems, understand new medical language, identify possible accommodations, reorganize daily life, and deal with uncertainty about what disability means for one's body, future, and identity [32, 66, 91, 106, 107, 115]. Prior work has explored important parts of this experience, including barriers to medical care, stigma and dismissal in healthcare, disability adjustment, and technology use in health-related contexts [8, 18, 63, 73, 84, 144].

The majority of the United States (U.S.) population reporting disability onset in adulthood (after age 21) [60], yet less attention has been paid to the early period of disability in adulthood. In this phase, diagnoses may still be developing, symptoms may fluctuate, and support needs may not yet be fully understood, resulting in delayed adoption of assistive tools and resources. Just as importantly, people may need support, accommodations, community, or other benefits that come with disability identification before they feel ready to identify as disabled or recognize their needs as disability-related [52, 86, 113]. This creates an important accessibility challenge: support is often needed before disability identity, care pathways, and technology relationships have settled into place.

In this paper, we explore this tension and accordingly investigate how adults use technology to navigate new disabilities and how emotional, social, cultural, and identity-related factors shape that process. We examine how people responded to constrained clinical encounters, how they assembled digital and social resources to fill gaps in care, and how disability identity influenced support seeking. Through semi-structured interviews with 14 adults who were diagnosed with a disability[1] within the past three years, we investigated the following research questions:

(1) How do newly disabled adults use technology to make sense of symptoms, diagnoses, and care pathways while navigating the U.S. healthcare system?

[1] Our recruitment process specifically called for recently disabled adults, although as our findings show, the disability label was still being negotiated by participants as part of their identity.

(2) How do barriers in the healthcare system influence patient[2]–clinician relationships and shape the form and extent of participants' technology use?
(3) How do disability identity, social, and cultural factors influence when and how participants seek support, accommodations, and disability-related technologies?

Our analysis contributes to accessibility and Human-Computer Interaction (HCI) research in three ways: First, we show how newly disabled adults with diverse symptoms and diagnoses consistently perform what we label as **credibility and coordination labor** across clinical and digital contexts to make sense of symptoms, pursue care, and advocate for themselves. Second, we show that technology use in this process is deeply relational: participants used digital tools to gather information and navigate trust, legitimacy, and communication within strained clinician contexts. Third, we show that disability identity, support needs, and technology adoption are not always synchronously aligned. Participants often sought diagnosis-specific tools, accommodations, and practical support while remaining ambivalent about, or even resistant to, disability as a label or category. We conclude with design and research opportunities for technologies that support people through transitional disability periods without assuming cohesive healthcare support or straightforward disability identification.

2 Related Work

Emerging symptoms and a new disability require engagement with the healthcare system on many levels. While clinical teams work hard to provide patients with care, institutional barriers and biases impact the patient journey. In this section, we overview these challenges on a broader healthcare scale, present struggles with technology adoption within these processes, and underscore our contribution of identity and labor-sensitive design considerations for future technology and research.

2.1 Barriers and challenges in process of seeking diagnosis and treatment

Levesque et al. (2013) defined healthcare access as *"the opportunity to identify healthcare needs, to seek healthcare services, to reach, to obtain or use health care services, and to actually have a need for services fulfilled"* [72]. However, financial constraints limit patients from obtaining or using health services in a timely manner. Black, Hispanic, low-income and under-insured individuals face longer diagnostic timelines due to financial constraints and language barriers [5, 33, 48, 54, 59, 109]. Insured individuals still face delayed care or give up medical treatment when the out-of-pocket payment is extremely high, causing negative long-term health consequences [50, 98]. Geographical location is another crucial factor for accessing healthcare. People living in rural or under-resourced areas may travel long distances to access healthcare facilities [31].

In a cross-sectional study investigating challenges in treating mental health, Coombs et al. (2021) found medical bill costs and the lack of a primary treatment location were the most predominant barriers for patients [23]. Limited availability for healthcare providers, specialists, and testing appointments also contribute to prolonging or delaying the timeline of seeking medical help and treatment [48, 54, 75, 135]. Shorter appointments were introduced to improve healthcare access, but clinicians and patients have both struggled to meet needs in such brief duration, resulting in incorrect evaluations and diagnoses, poor patient experience, and prolonged medical processes [131]. Our study further identifies how these temporal tensions shape the experiences of newly disabled adults and the technological resources they leverage when symptoms are dismissed or diagnosis and treatment are not readily available.

A predominant barrier documented in the medical system is gaslighting, a recently coined term to capture patient concerns being inappropriately dismissed and invalidated, and instead attributing symptoms to emotional factors, a person's identity, or their lifestyle [14, 38, 41, 94, 96, 117]. Prior research has demonstrated that women in particular are disproportionately affected by medical gaslighting in healthcare settings [6, 15, 22, 34, 45, 61, 67, 68, 92, 99, 117, 126, 133, 138, 148]. For example, Merone et al. (2022) [87] discovered that women with chronic conditions struggled to have their pain taken seriously despite it significantly impacting their daily lives, and were often treated with psychological support instead of medication [116]. Many women reported traumatic feelings of stigma, shame, and blame after being gaslit by healthcare professionals, leading them to be afraid of accessing health services [87]. Medical gaslighting is also more commonly found among female patients with medically ambiguous or unexplained syndromes like ME/CFS (Myalgic Encephalomyelitis/Chronic Fatigue Syndrome) and POTS (Postural Orthostatic Tachycardia Syndrome) [87]. Studies have also found other marginalized communities, such as Black [6, 15, 139, 142] and Queer patients [56, 96, 101], to be disproportionately affected by medical gaslighting, emphasizing roots in epistemic bias in healthcare and pain management [19, 29, 49, 51, 114]. Our findings further contextualize participant experiences with medical gaslighting within early disability experiences and highlight the ways in which participants build credibility through extensive research and labor.

2.2 Emotional and mental health post disability diagnosis

Tajfel and Turner's Social Identity Theory proposed that affirming disability identity protects the well-being of people with disabilities [132]. This theory is complemented by several studies suggesting that self-esteem and disability acceptance are linearly correlated [63, 84]. Shakespare (2006) and Boardman (2020) characterized how self-acceptance is mediated by the time of disability onset: people who became disabled later in life faced a more sudden and complex process of transitioning from their previous able-bodied identities to disability identities and dealt with varying degrees of loss [7, 118]. People with congenital or early onset (or longer condition duration) thus tended to have greater disability self-concept and acceptance, hence greater satisfaction with life, self-esteem, self-efficacy and income [8, 18, 63, 73, 144]. Experiencing a new disability later in life is also commonly accompanied by a grieving process where people experience emotions including shock, fear, confusion, anxiety,

[2]We acknowledge that "patient" has been used in the medicalized definitions and models of disability. However, our use of this term does not refer to participants' disability or identity [77]. Rather, we use "patient" throughout this paper to refer to all our participants actively in the process of seeking medical care.

denial, and self-blame [47, 82, 90, 95, 125]. These nuanced facets of disability acceptance in adulthood, as opposed to congenital disability, motivate us to further investigate the early disability and acceptance process for adults and how this process influences the tools and resources patients choose to interact with.

2.3 Challenges adopting assistive technologies in early disability experiences

Yeung et al. (2016) uncovered that the average use of assistive technology (AT) by people with disabilities is lower than their typical needs, suggesting low AT adoption or abandonment [136]), despite potential benefit [146]. Barriers to using AT can be informational, psychological, or societal [55]. Major factors can include the level of difficulty adapting to AT, perceived benefits and assistance provided by AT, and self and societal perceptions on disability [16, 55, 80, 91, 103, 112, 140]. Motahar and Wiese (2024), after working closely with spinal cord injury patients and rehabilitation clinicians, pointed out that adopting new AT after injury lies in patients' willingness to accept that their condition is long-term, which makes them people with disabilities instead of just patients [91].

While these papers take a more medical approach, HCI research has similarly investigated AT adoption. For example, Hurst and Tobias (2011) advocated for an empowering do-it-yourself approach to designing AT, suggesting that a form of ownership over AT can improve adoption [57]. An HCI lens has also uncovered how social, professional, and environmental factors influence AT adoption [10, 123]. In our study, we extend this HCI approach to an understudied population (newly disabled patients). We investigate how the early facets of disability identity formation build perceptions of AT. We also identify sociotechnical influences on technology adoption and capture how participants mentally frame the technology they are comfortable using and implications of this for future technology design.

2.4 Disability diagnosis, information, and community building through social media

Social media plays a meaningful role before and after diagnosis. Disability-related content made by people with disabilities on social media platforms has been found to help people recognize themselves and seek diagnoses [35]. People also appreciate first-hand, shared experiences posted on social media, as this information may not be fully captured by existing research studies and medical websites [35, 124]. However, there are also risks associated with viewing or engaging in online disability-related content. Particularly among young people, this content tends to be used for self-diagnosis or can grow a belief that one has undiagnosed mental health conditions [3, 25, 39]. Previous research claims that youths lack the medical expertise to distinguish between stress or mood fluctuations and severe disorders, causing unnecessary distress and anxiety through self-fulfilling prophecies [30, 104]. Such self-diagnosis may also reinforce stigma and misconceptions of mental health conditions, since online content that youths used to self-diagnose may contain inaccurate or incomplete information [30]. Our findings and discussion raise new tensions between the benefits of lived experiences and medical information on social media with the risks of misinformation within the unique context of new disability navigation. We also pose design considerations around social media as part of formal patient research processes.

3 Methodology

3.1 Participants

To understand the process of navigating a new disability, we interviewed 14 adults (9 female, 4 male, and 1 non-binary) who reported newly experiencing or receiving a diagnosis relating to a disability within the past three years. All participants were English speakers and based in the U.S.

During recruitment, we invited participants who identified as *"having any disability"* to complete our screener survey. Eligible participants answered the prompt *"How would you describe your disability?"* in the survey, even though many expressed uncertainty about disability identity later during interviews, further illustrating how disability may hold different meanings depending on context [86]. We prioritized recruiting participants who with sensory disabilities, mobility disabilities, cognitive disabilities, and chronic illness. Many participants self-reported multiple disabilities, including neurodivergence (i.e., Autism, ADHD), when asked about their disability in the screener survey. While we discussed how neurodivergence intersected with receiving medical care, we did not focus our interview on this aspect, as participants described exhibiting neurodivergent symptoms since childhood.

Participant information is detailed in Table 1. As some participants had rare conditions, we abstracted participant disabilities, diagnoses, and conditions to broader descriptions throughout the paper and in the participant table to maintain anonymity.

3.2 Recruitment

We initially recruited participants through disability-focused social media groups (including Facebook groups and subreddits), email lists from disability organizations in the U.S., and User Interviews [3]. Our screener survey asked for contact information, age, gender, and occupation. We also asked participants to describe their disability, how recently they were diagnosed or started experiencing symptoms, and how their daily life has changed since. We asked participants about daily technology use with tools such as artificial intelligence (AI) chatbots, smartphone applications, and smart home devices. To help with organizing and filtering potential fraudulent participants, we asked through which channel people had found our screener survey.

However, from these methods, we received many bot and fraudulent responses in our screener survey. To ensure participant and data authenticity, we implemented multiple different approaches to verify our participants, such as identifying suspicious patterns across responses and conducting follow-up conversations with potential participants over email and/or video chat to confirm occupation, recruitment channel, and other shared information.

[3] An online platform that connects researchers with vetted individuals who participate in paid studies.

Table 1: Details of Interview Participants

Participant ID	Gender	Age	Recent Diagnosis	Symptom and Diagnosis Timeline
1	Female	46	Neuropathy	Started experiencing symptoms early 2024. Diagnosed June 2025.
2	Non-binary	27	Chronic shoulder and joint pain	Work accident injury leading to disability in 2024.
3	Female	27	Auditory processing disorder	Lifelong symptoms but did not notice issues until 2020. Sought and received diagnosis in 2025.
4	Female	34	Chronic illness, post-traumatic stress disorder, anxiety	Started exhibiting symptoms in 2022. Diagnosed in 2023.
5	Male	51	Stroke related mobility impairment	Stroke in October in 2022. Symptoms returned summer of 2023 after cancer relapse.
6	Female	52	Stroke related aphasia and vision loss	Stroke in November 2023.
7	Female	51	Chronic inflammatory condition, Chronic autoimmune disorder	Diagnosed across 2023 and 2024.
8	Female	53	Lower-body paralysis	Surgery in October 2022. Paralyzed as a result of surgery
9	Female	57	Fibromyalgia, arthritis, osteoporosis	Started seeking diagnoses in 2021. Approved for social security disability in 2023.
10	Male	34	Mobility impairment	Work accident August 2022. Diagnosed as herniated disc and mobility impairment May 2023.
11	Male	67	Shoulder/arm mobility impairment	Cancer related surgery led to disability in April 2024.
12	Female	38	Connective tissue disorder	Lifelong symptoms but did not anticipate it was disability-related until symptoms worsened recently. Suspected diagnosis March 2025 and confirmed May 2025.
13	Male	24	Multiple Sclerosis	Started exhibiting symptoms summer 2022. Diagnosed 2 months later.
14	Female	39	Gastrointestinal condition	Symptoms on and off 2020-2024. Diagnosed March 2025.

3.3 Interviews

Interviews were semi-structured and approximately 75 minutes long. All interviews were conducted on the video conferencing platform Zoom. At the start of the interview, we confirmed that the consent form was signed and once again asked for verbal consent to audio and video record the interview. This study was approved by the appropriate Institutional Review Board. All participants were compensated with a $30 visa gift card.

During interviews, participants were asked about their disability symptoms and diagnosis process, healthcare timeline, emotional processing, support systems, social/cultural stigma, resource and technology use, lifestyle changes, health management strategies,

and identity formation. We also asked participants about their desires for technology development related to their disability. The interview guide is available in Appendix A.

3.4 Data Analysis

3.4.1 Research Team. Data was collected and analyzed by a research team consisting of three undergraduate student researchers at a liberal arts institution led by a faculty PI and mentored by a faculty collaborator. The undergraduate researchers brought interdisciplinary perspectives from Media Arts and Sciences, Computer Science, and Cognitive and Linguistic Sciences, and were trained in qualitative HCI methods including semi-structured interviewing, thematic analysis, and accessibility- and disability-centered design frameworks such as interdependence [4], inclusive design [53], design justice [26], and contrasting models of disability [42, 44, 119]. The PI and faculty collaborator have approximately eight years of experience conducting accessibility research using qualitative methods to understand the experiences, expertise, and needs of diverse populations.

3.4.2 Coding Process. All interview recordings were de-identified by a research team member, and transcribed using the software Trint (each transcript was verified by a research team member). Data were analyzed on FigJam and Miro by four of the authors through inductive thematic coding, involving a process of affinity diagramming, pattern identification and clustering, and theme refinement [2, 11, 141]. The team collaboratively analyzed and coded the first five interview transcripts. They organized participant data as a rough timeline: how participants navigated initial symptoms and diagnosis, lifestyle changes and navigation after formal diagnosis, cultural and social factors influencing their journey, identity negotiation, and technology use/needs. Then, the team took another pass through participant data to note dominant and interesting themes within and between these categories, such as the emotional and cultural processes of early disability (example code: *coming to terms with a chronic condition*), how specific technology supported their journey (example code: *AI use to navigate mental fatigue*), and negotiations of identity (example code: *finding validation on social media while having an invisible disability*).

After creating this shared baseline, authors split up and coded the rest of the transcripts individually, while routinely meeting as a group to iteratively refine codes by broadening, narrowing, combining, or splitting codes. Authors found consistency through group discussion and memoing on FigJam during analysis to converge on interpretations of the codes. As an example, an initial code on "*relationship changes with disability through progression*" was broken down into separate codes defining relationships in the initial and later stages of disability symptoms/diagnoses to better capture processes around acceptance, identity, and resulting technology adoption. Finally, these codes were grouped into higher-level categories which comprise the findings in this paper.

3.5 Limitations

Due to our struggles with finding real participants amongst bots and frauds [102], we prioritized certain factors (diagnosis within past three years, adults over age 18) while building our participant pool. Our recruitment process resulted in an over-representation of women in our participant pool (9 out of 14). In this early stage of research capturing the experiences of newly disabled adults, we aimed to study a small number of disabled participants in-depth using qualitative methods to detail the challenges and emerging strategies of this group. We also included a wide range of disability (sensory, mobility, cognitive, and chronic illness), rather than focusing on a specific diagnosis or category. We believe our findings are strengthened by showing consistent experiences in the healthcare system across this range [76, 78]. However, it is important to note that we do not consider our participants as a uniform group. Throughout the paper, we provide participants' disability conditions in parenthetical or narrative form to help contextualize how their specific disability influenced their experiences. We do not interpret our findings as generalizable to the broad population of newly disabled adults. Rather, our analysis offers a preliminary look into the strategies used and required among diverse newly disabled adults and presents avenues for future work with more granular identities and subpopulations.

Participants described experiences shaped by gender, culture, and neurodivergence. In this work, we discuss neurodivergence when participants connected it to their ability to access healthcare, such as clinical bias in the medical system. We took note that medical gaslighting was consistent among female participants, in line with prior research. We also noted when participants described how specific cultural or race framings ingrained certain beliefs towards disability. However, gender, race, culture, and neurodivergence were not primary focuses of our research questions nor the lens through which we focused data analysis, and therefore our dataset does not support broader claims regarding these factors. Future work must investigate how these intersecting identities further shape required labor, emerging disability experiences, and resulting technology and resource use.

This study specifically focuses on the U.S. healthcare context, characterized by high out-of-pocket costs, market-driven healthcare financing structures, and complex federal and state regulations [1, 85, 111, 134]. As such, findings should be interpreted within this national context and may not be applicable to healthcare systems that differ structurally in their organization and operation.

4 Findings

Participants described navigating recent disability through clinical, digital, and social systems that rarely fit together neatly. In our data, we observed three recurring sociotechnical tensions. First, participants often had to establish credibility in medical contexts where their symptoms, pain, or diagnoses were doubted or compressed into short appointments. Second, they responded by piecing together fragmented care and information support infrastructures using digital tools. Third, participants' relationships to disability, diagnosis, and support were often in flux, shaping when and how they sought accommodations and disability-related technologies.

4.1 Building Credibility Under Constrained and Dismissive Care

With understaffed clinics, overbooked clinicians, and insurance policies timeboxing medical appointments, participants described navigating a medical system that felt restrictive and misaligned

with the complexity of what they were experiencing. Barriers in this process included delayed referrals, long waits between appointments, difficulties finding specialists, and limited opportunities to explain evolving symptoms in detail [5, 33, 50, 131]. While these barriers are not unique to disability, participants described them as particularly consequential while trying to secure support as their symptoms progressed and posed a potential for permanence.

Disabilities often had fluctuating symptoms, week-to-week and month-to-month. P12, who was recently diagnosed with a connective tissue disorder, described how her disability regularly manifested in multiple overlapping bone displacements and resulting injuries. These frequent changes within her body caused major updates between clinical visits that were difficult to communicate in short appointments, even when she compiled and printed summarized handouts as an aid:

> "It's hard these days to get a doctor to look at something or spend time. They are 15 minute appointments and you can't tackle [connective tissue disorder] in 15 minutes. You go in for one issue. They don't want to deal with multiple issues, they only want to deal with one issue. But if it's [connective tissue disorder], it's like, okay my fibula keeps dislocating. I also have a problem with my sacrum, I also have a problem with my left hand, and technically they're all related...So trying to get all of that across in 15 minutes, it's hard. You hand them a piece of paper, but they've got like 16 other people to see, if not more. Are they gonna read that piece of paper? Sometimes. Good doctors will. They will find the time, they will make the time. They will take the paper to the bathroom with them if they have to. But most doctors I would not put in the category of good doctors."

P12's distinction between clinicians who *"find the time"* and those who do not illustrates that participants valued more than clinical expertise alone. Participants described supportive clinicians as those who spent time understanding the complexity of their symptoms and validating their lived expertise while developing care plans. These moments resemble Mingus' concept of access intimacy, in which access is experienced through relationships that recognize and accommodate disability rather than those which require people to continually justify themselves [88, 89].

Getting appointments with primary care doctors and specialists in a timely manner was also challenging, with P7 (chronic inflammatory condition and autoimmune disorder) explaining that the dynamic nature of disability and chronic illness was at odds with long wait times: *"Doctors are freaking overbooked as heck. I have an appointment with my doctor that I scheduled for blood work done in March. This fool can't see me until August. The blood work has changed by then!"* In some cases, these time constraints came from employment structures that controlled access to medical care. For example, after a cancer-related surgery, P11 (shoulder/arm mobility impairment) lost mobility in the upper right side of his body and was supposed to receive immediate physical therapy through the U.S. Veterans Affairs Department (VA) within 30 days. However, the VA was not able to connect him with a physical therapist for 9 months, at which point *"the damage was done"* and his lack of mobility became permanent. Participants' experiences also reflected the temporal mismatch that Kafer (2013) describes through *crip* time, in which disability disrupts normative assumptions about how bodies change, recover, and fit within institutional schedules [64].

Participants, especially women and those with histories of chronic illness or mental health diagnoses, frequently described encounters in which clinicians minimized their symptoms, attributed concerns to anxiety or stress, or did not take their interpretations seriously. Several participants used the term *"gaslighting"* to describe these experiences [6, 9, 15, 22, 45, 61, 67, 68, 92, 99, 117, 126]. As an example, P14 (gastrointestinal condition) explained that doctors did not want to run further tests on her when she first experienced symptoms due to her age, gender, and use of antidepressants. She was instead diagnosed with anxiety, which she attributed to being a *"fast talker"*, *"fitting the bill"*, and the way she presented herself to clinicians. She avoided further treatment and deferred to medical professionals, explaining *"I'm not above thinking it's mental illness...I'm not the doctor."*. When she continued to lose weight, among other physical symptoms, her friends urged her to seek a diagnosis again. When she was finally diagnosed with a rare gastrointestinal condition, she felt relief and validation that her illness and symptoms were real: *"I feel so good about not being crazy."*

However, dismissal occurred even when participants had no documented history of mental illness or neurodivergence. P6 (stroke-related aphasia and vision loss) described her medical journey as well-resourced and supported, yet it still took five weeks for doctors to diagnose her sepsis after initially brushing off her symptoms as the flu. By then, the infection had caused a stroke that led to vision loss and mobility impairment, requiring extensive physical therapy.

Time constraints and gaslighting shaped how participants approached clinicians. Rather than entering appointments expecting collaborative exploration, many prepared for how to communicate health information quickly to avoid being dismissed or redirected. Participants described bringing partners or family members to appointments, documenting symptoms over time, compiling prior test results, printing research articles, and rehearsing how to phrase concerns in ways that might be taken seriously. Some referenced their educational or professional background to establish credibility. P9 (fibromyalgia, arthritis and osteoporosis) described having to become *"a full-time patient"* just to keep up with the level of research, organization, and self-advocacy needed to move through the healthcare system. Others described similar efforts to present themselves in ways clinicians would not dismiss as vague, anxious, or unreliable.

Participants not only managed symptoms, but also managed how those symptoms could be made intelligible within constrained clinical encounters. They were often expected to translate dynamic, interconnected, and evolving experiences into concise, actionable narratives. When these efforts failed, the consequences included delayed diagnosis, deteriorating trust in clinicians, and, in some cases, irreversible progression of disability. Faced with these gaps, participants increasingly turned to digital tools, online resources, and peer networks to help fill in what formal care could not provide.

4.2 Piecing Together Digital Support Beyond the Clinic

Barriers in the medical system often left participants with unanswered questions, delayed support, and uncertainty. Participants were also trying to understand how people realistically lived with their disability and where to begin finding resources. These needs required participants to move across clinical, digital, and community sources of information in ways that often felt time-consuming and exhausting.

4.2.1 Using digital tools to research, coordinate, and prepare. To compensate for short appointments, long wait times, and credibility challenges, participants frequently described doing extensive self-directed research between appointments to learn more about their new disability, potential causes, medications, and treatments. Participants searched for symptom and disability information on the Mayo Clinic, Cleveland Clinic, diagnosis-specific organization websites, government health websites, university health sites, and academic papers found through Google searches. They also had to break down and understand health information from patient portals, such as visit summaries, lab results, and medical histories. Over time, many participants developed structured practices for managing the volume and complexity of information they encountered. This included saving articles, organizing notes, and creating spreadsheets.

Participants' research often kept care moving, with P3 (auditory processing disorder) conducting over 300 hours of internet research when learning about her disability. P9 (fibromyalgia, arthritis and osteoporosis) used the aforementioned sources to learn about doctors in her area, tracking them in a spreadsheet she created as she built her care team of a primary care doctor and 15 specialists. P12 (connective tissue disorder) identified both her diagnosis and the root cause of her suddenly worsening symptoms by reading research abstracts in online medical journals, determining which articles were worth paying for, and summarizing information to present to multiple clinicians as a potential diagnosis. Similarly, once P10 (mobility impairment) understood the result of his workplace injury (a herniated disc), he researched potential treatments online, developed his own treatment plan and presented it to doctors, asking for their approval to move forward. P14 (gastrointestinal condition) combined the medical MyChart system with her online research to interpret and monitor her vitals and blood work post-diagnosis. She explained that with continued care, there are *"so many variables that I have to constantly keep up with to keep myself afloat"*, adding, *"I basically have to pretend like I'm my own doctor 90% of the time."* Many participants echoed the labor of keeping track of multiple parts of their disability and often intersecting disabilities, with P3 (auditory processing disorder) and P14 (gastrointestinal condition) suggesting empathy-driven tracking tools to help keep tabs on disability and symptom changes over time. This digital research process also had barriers. Academic research was often gate-kept behind paywalls and inaccessible jargon. Participants explained there was no starting point for research. They further labored to find sources, determine source credibility, and wade through massive amounts of information.

Participants described tension when bringing information they had researched and compiled to doctors. As P7 (chronic inflammatory condition and autoimmune disorder) recalled, clinicians would say, *"Don't let your WebMD or Google medical degree get in contact with my medical degree."* This tension influenced the sources participants turned to when searching for and summarizing information to bring back to their clinician. P7 (chronic inflammatory condition and autoimmune disorder) continued *"I'm reading the same thing you are reading. I'm gonna look at the medical books that you're looking at. Because I'm highly intelligent. You're not going to gaslight me and tell me something's wrong with me and it's not wrong with me."* Participants were also keenly aware of biases and accordingly sought information from medical and research foundations. However, P4 (chronic illness, post-traumatic stress disorder, anxiety) noted that even research papers could have homogeneous and unrepresentative participant pools. Instead, working in the disability field, she relied on information from her community support system.

Participants were going well beyond simple internet searching. They were constructing research infrastructures across multiple digital resources to navigate phases of early disability experiences. In the absence of coherent guidance, they became researchers, organizers, and coordinators of their own care. This process was also shaped by tensions around bringing information to clinicians, influencing which sources participants found viable, further reflecting the labor involved in building credibility with clinicians.

4.2.2 Turning to lived experience for practical and emotional support. While institutional medical resources were important for understanding diagnoses and treatment options, participants often turned to online communities and social media for forms of support that clinical encounters did not provide. These spaces helped participants understand how others experienced the same symptoms day-to-day, what adaptations were helpful, what treatments felt worthwhile in practice, what it meant to live with disability over time, and how to grapple with the resulting emotions (participants stated feelings of *fear, grief, anger, isolation, frustration, anxiety, confusion, curiosity, acceptance, and relief*). Participants sought out diagnosis-specific Facebook groups, Reddit communities, Instagram accounts, YouTube videos, and other online spaces where disabled and chronically ill people shared lived experiences.

"Lived experience is worth gold", P14 (gastrointestinal condition) explained while describing how Reddit became a crucial platform in connecting her with other people who share her rare condition, which disproportionately affects women. Disability-specific subreddits, especially for rare disabilities, offered both community and practical advice for participants, such as P2 (chronic shoulder and joint pain) learning to navigate workers' compensation for their frozen shoulder and P5 (stroke-related mobility impairment) looking up emerging symptoms to find if other stroke survivors had experienced something similar. P7 (chronic inflammatory condition and autoimmune disorder) described similar content on disability-specific Facebook groups, including advice on everyday living adaptations, navigating the medical system, and communicating/advocating with doctors. Participants also used social media to vet doctors, with P2 (chronic shoulder and joint pain) using Facebook to determine which specialists would not be transphobic.

Beyond practical advice, Reddit provided validation, space to vent, and reduced feelings of isolation. P13, recently diagnosed with Multiple Sclerosis (MS), found that hearing shared experiences *"made things feel like less of an individual burden"* and validated his own experience living with a currently invisible disability. He elaborated:

> "MS is, I like to think, a relatively invisible condition. In everyday activities, it's hopefully not going to show up too much. Which is usually really great, but it can also feel isolating...It's a solitary experience for that reason, cause it's not really visible. Hearing other people share, especially success stories, but not exclusively, just hearing people share that they might've had a similar symptom or a similar experience, with like paper shopping bags or something that I did can definitely help reduce that feeling of isolation."

P5 (stroke-related mobility impairment) similarly described feeling *"less alone"* when reading shared experiences of stroke and cancer recovery. He, like many others, highlighted the power of *"commiserating"* on social media and how it could open dialogue between users: *"I can try to help them, or I'm going through something new and they can help me."*

However, some participants avoided social media over concerns about bias and misinformation. Others described how these platforms could also reproduce broader social harms. P12 (connective tissue disorder), for example, highlighted how the prevalence of misogyny on Reddit deterred her from using the platform. While disability subreddits tended to be a more *"benign"* part of the platform, she still encountered ableist content deeply rooted in societal stigma, with comments accusing posters of *"whining"*, questioning the severity of disability symptoms, and telling women they *"deserve some of these afflictions"*.

Within their broader support infrastructures, participants sought and provided information, practical advice, and experience about living with a new disability, highlighting Reddit and Facebook as especially valuable. These technologies supplemented formal and in-person support by offering lived experience and validation of pain, symptoms, and healthcare barriers.

4.2.3 Cautiously using AI in care support. A smaller but notable group of participants described using AI tools, particularly chatbots, to make sense of their experiences between appointments. These tools were typically used as intermediaries for organizing or interpreting information that participants had already gathered, rather than as a tool for new information seeking. Participants described using AI to summarize research articles, explain medical terminology, interpret test results in simpler language, brainstorm possible questions for upcoming appointments, or compare information across multiple sources. In these cases, AI lowered the barrier to engaging with complex or technical information and reduced friction in understanding and preparing research.

However, participants expressed concern about privacy, the risks of entering sensitive health information into AI systems, accuracy, and over-reliance. There were also credibility concerns: even when AI helped participants prepare privately, some were hesitant to reference AI-informed insights during appointments to clinicians. These concerns extended beyond the clinic, with participants also fearing judgment from friends and family.

While most participants who used AI leveraged it for informational sensemaking, P14 (gastrointestinal condition) highlighted a unique case of how chatbots supplemented her emotional support systems. She trained ChatGPT to speak to her in a way that communicated crucial disability information without being too solutionist and avoided certain words that incited panic about the future direction of her disability (such as *"stroke"*). She also used ChatGPT for validation, similar to how other participants used Reddit and Facebook, with it providing comfort after she described situations in which she felt judged due to her disability. She described the chatbot as therapeutic; It did not interrupt her, and she preferred the tool in crisis moments based on poor encounters with other mental health support structures (e.g., the crisis hotline). However, P14 was cautious and aware of the dangers of becoming too reliant on chatbots, clarifying that she will always continue to see an in-person therapist.

AI was treated as a useful information synthesis aid (and in one case, emotional aid) when formal care was incomplete, but use came with considerations around social judgment and credibility. Participants' use of AI reflected the same pattern seen throughout this section: technology became most valuable when it helped bridge the informational, interpretive, and logistical gaps left open by healthcare systems.

4.3 Negotiating Disability Identity and Related Support

Despite our recruitment and interview materials using disability-oriented language, participants rarely described disability identity as something they immediately or easily embraced. Some were surprised by the question of whether they identified as disabled, while others described still deciding how, when, or whether that label fit. Many occupied an in-between space: they were navigating disability as a lived experience while uncertain about disability as a category or identity. This ambiguity shaped what kinds of tools, accommodations, and communities felt accessible or appropriate.

4.3.1 Culture and upbringing shaping early views of disability. Participants described how religion, culture, upbringing, and family values shaped what disability meant to them long before they experienced disability themselves. These early beliefs often influenced whether disability was considered worthy of discussion, support, or accommodation, and in turn shaped how participants later approached assistive tools and other forms of support. For example, P1 (neuropathy) discussed how Catholicism in her family often stifled feelings and pushed conversations around disability under the rug. P3 (auditory processing disorder) similarly grew up in an Italian, Catholic family with parents who *"don't believe in doctors"* and a culture that did not emphasize women's health. P3 admitted that these stigmas had given her a *"narrow-minded view of disability"*. Participants from Asian backgrounds described similar dynamics. P4 (chronic illness, post-traumatic stress disorder, anxiety) explained that disability was not necessarily celebrated or openly discussed within her community, while P5 (stroke-related mobility impairment) described how this *"Asian mentality"* led him to avoid using his disability parking pass for a long time.

Upbringing also shaped how participants interpreted resilience, independence, and what kinds of support were justified. P12 (connective tissue disorder), who described herself as a *"military brat,"* explained that she had been raised to *"head down and power through just about anything, even if we shouldn't."* Although her support system would help with practical tasks when needed, she described a broader *"pull yourself up by your bootstraps"* mentality. In her experience, health issues were only considered worth discussing under extreme circumstances, such as amputation or risk of death. As a result, she delayed seeking accommodations and assistive tools, viewing them as *"special treatment"* or a shortcut.

Participants often held deeply ingrained beliefs about what counts as disability, who is entitled to support, and what kinds of help are acceptable to ask for [52, 113]. These beliefs continued to shape whether disability-related tools and accommodations felt appropriate.

4.3.2 Support, grief, and the threshold of permanence. Participants were processing the permanence of their disability, with many initially believing that their symptoms were temporary. P2 (chronic shoulder and joint pain) believed their workplace injury would resolve in 6 months through physical therapy, but re-classified their injury as chronic pain after a prolonged timeline. P5 (stroke-related mobility impairment) underwent physical therapy after a stroke, improving for a while. However, when his cancer relapsed, his mobility impairment concurrently returned, requiring significant lifestyle changes. Similarly, P6 experienced vision loss after her stroke, but hoped this was a side effect and she would regain vision over time. Many viewed disability as something hopefully temporary.

P12 (connective tissue disorder) explained how this acceptance of a permanent disability could feel like a *"grieving process, because of the expectations and ideas that you have had. You feel a loss for that."* P5 (stroke-related mobility impairment) and P11 (shoulder/arm mobility impairment) similarly described grieving the life they thought they would have, reflecting Corbin and Strauss's (1985) concept of biographical work, in which chronic illness prompted people to reconstruct their identities and expectations for the future [24].

Participants reflected that not all technologies felt emotionally compatible with where they were in the acceptance process. For example, one participant described trying meditation apps and positivity-focused social media content to cope, but found that such messaging did not resonate while she was feeling resentment, anger, and grief. These examples highlight how timing and emotional fit mattered: technologies that might seem supportive in theory did not always align with what participants were ready to engage with in practice.

Participants sometimes delayed engaging with disability-related supports because using them felt like acknowledging a new baseline or a changed future. P11 (shoulder/arm mobility impairment), for example, had lived for decades with a military injury without identifying as disabled, because he could still live *"independently"*. After a cancer-related surgery led to new mobility limitations, he resisted accommodations and assistive tools because he believed his body would improve. He reflected, *"I probably should [use technology], but it is hard to switch your whole way of thinking, where you have always been able to do stuff and now you can't."* He continued on the difficulty of acceptance, saying:

> "Getting old ain't easy, and accepting that the disabilities which you are hoping are temporary are gonna be permanent isn't easy. Once you accept it, it kinda becomes permanent. So as long as you don't accept it, maybe it is still temporary...if it doesn't disable you, are you disabled?"

Only when he began to feel that he could no longer live without support did he start using assistive tools such as a fall alert and considering physical adaptations in his home. Other participants similarly described resisting or postponing accommodations until the practical reality of disability became harder to defer. In this way, technology and accommodation use became entangled with perceptions of permanence and individual disability timelines.

4.3.3 Defining disability relationally. A major influence on disability identity formation was how participants compared themselves to other disabled people. This was especially common among participants with invisible disabilities, who often struggled to identify themselves as disabled when others around them had more visible or seemingly higher-need disabilities. P3 (auditory processing disorder) explained this hesitancy: *"I didn't want to be disrespectful to people who have disabilities that can't ever be hidden. To me, it felt like almost an insult to be like, yeah, we are in the same category. Maybe someone needs a lot more support and a higher level of care."* For participants like P3, disability was not a neutral descriptor. It was associated with visibility, severity, and resource allocation.

Participants also reflected on how race, privilege, and public perception shaped these comparisons. P3 (auditory processing disorder) noted, *"white people love to claim oppressed identities whenever they can, and I think that was something I didn't want to fall into,"* highlighting how broader social narratives influenced her discomfort with publicly aligning with disability. Using accommodations or assistive technology in public could feel inappropriate or attention-seeking. Work also shaped these interpretations. Several participants did not associate themselves with disability because they could still live independently, maintain employment, or avoid visible supports and accommodations, even while navigating chronic pain, fatigue, or mobility limitations.

P10 (mobility impairment) reflected this tension clearly. He explained that *"there are people that have a lot more disability that should have more attention or resources than somebody that has a herniated disc."* He did not describe himself as part of the disability community because he did not use mobility aids or assistive devices, yet later discussed wanting a spinal compression-related support tool that would help his chronic pain. This highlighted a recurring tension in our data: participants could desire and benefit from disability-related tools while still feeling uncertain about whether disability, as a broader identity category, applied to them.

For some participants, diagnosis-specific identities felt easier to inhabit than disability more broadly. P13 suggested that labels such as *"chronic illness"* or diagnoses such as *"Multiple Sclerosis"* felt more precise and less charged than disability as an umbrella term. Many participants were actively defining their relationship to disability as a term and community, while still readily using

diagnosis-specific or healthcare system tools such as MyChart, physical therapy platforms, and online medical resources.

Our findings reveal a complicated relationship between disability identity and support uptake. Technology use was sometimes treated, by participants or by others, as evidence of being disabled, even as participants desired the benefits it could provide. Diagnosis-specific framing often felt more comfortable and less politically or emotionally loaded than disability as a broader identity. Navigating new disability involved renegotiating one's relationship to disability itself by deciding which labels felt meaningful, what kinds of support felt available, and what futures felt imaginable. Participants' relationships to technology were then shaped by this process.

5 Discussion and Future Work

Across participants, new disability was experienced as an ongoing process shaped by constrained clinical care, contested diagnoses, fragmented digital resources, and evolving negotiations with identity and support. Participants were learning to manage symptoms, present themselves as legible and credible to clinicians, assemble care information across platforms, and decide whether disability-related tools, accommodations, and communities applied to them. We discuss the implications of our research for future technology design and research aimed at supporting patient labor, communication, and care navigation.

5.1 New disability as a sociotechnical process entangled with identity and grief

Building on prior work showing that disability identity is contextual, relational, and continually negotiated rather than fixed [35, 52, 86, 113], our findings examine an earlier phase of this process, when participants were making sense of bodily change while pursuing diagnosis and determining whether disability and its associated communities, accommodations, and technologies applied to them.

Many of the technologies participants relied on before they had necessarily incorporated disability into their self-concept were not traditionally framed as accessibility technologies, but as general-purpose tools repurposed into disability support infrastructures [58]. This included search engines, patient portals, spreadsheets, online forums, messaging platforms, and AI systems. Some readily used diagnosis-specific or medically aligned resources (e.g., diagnosis-related social media pages, health clinic websites, and health tracking applications) while still negotiating identification with the term *"disability"*.

Participants' relationships to accommodations, assistive tools, and disability-related technologies were shaped by grief, stigma, self-perception, permanence, and uncertainty about whether disability was a category that applied to them [17, 108, 113]. Some compared themselves to people they perceived as higher need, often perceived as having more visible or severe disabilities. Despite requiring support, these comparisons led some participants to view assistive tools as inappropriate given their current level of independence or even other sociodemographic factors, such as race. These examples highlight relationships between comparative identity and resource adoption, with participants hesitating to integrate accommodations and assistive tools into their care practices while they negotiated perceptions of others and themselves.

5.1.1 Research Implications. Our findings point to a gap in the disability navigation process that accessibility and disability design could support: the period when people are still gathering evidence, coordinating fragmented care, and trying to understand what kinds of support might fit into their lives. Research that focuses only on already-recognized disability risks overlooking and supporting a large portion of the work participants described. We also caution against treating the general-purpose digital tools participants leveraged as incidental [100]. Rather, future work should examine how everyday digital systems become part of disability support infrastructures that help patients research symptoms and diagnoses, communicate with clinicians, and implement lifestyle adaptations to a new disability.

Identity negotiations also highlight a gap in how we translate identity and community into research practice, particularly in recruitment language, study design, and recommendations. Calls for community-in-the-loop practices and community-led technology development raise a key question: How do we define these communities when disability identity itself may be fluid, resisted, or still emerging? If tools are framed through identity, could they become symbolically tied to permanence, loss, or public alignment with a category that feels risky or misaligned depending on the sociopolitical climate? These tensions raise practical research issues as well, such as how we frame recruitment calls [35]. Could seeking disabled people for research studies exclude people who still align with the experiences we aim to highlight and who may benefit from the technology we aim to create? Do our accessibility recruitment calls further exacerbate the biases of who self-selects into research? Future work should explore how to define inclusion criteria, frame tools broadly or narrowly enough to meet people where they are, and draw from justice-oriented scholarship that treats identity as fluid rather than fixed [20, 35, 37].

5.1.2 Design Implications. Identity assumptions shape access to technologies framed explicitly around identity. If tools are framed only around specific disability identities, such as visual feedback applications often targeting blind users, others may not see these tools as meant for them as they navigate emotions around the permanence of their new disability symptoms (such as P6 potentially benefiting from visual feedback tools after her stroke, while still hoping that she would regain vision before being categorized as blind). Previous research has also shown that technologies which visibly signal disability may be isolating or difficult to adopt [16, 36, 65, 121]. While many individuals proudly identify with disability as a critical part of their identity and values, our participants showed that people may need support, accommodations, and tools long before they are willing to adopt the label (or may never adopt it at all) [52, 113].

Designing for this reality means creating systems that do not require users to readily identify as disabled in order to benefit from support. One implication is the value of flexible entry points. Participants often found diagnosis-specific communities, symptom-based resources, or practical adaptation tools easier to engage with than disability-framed ones. This suggests that support technologies may be more approachable when framed through immediate

needs, lived problems, or condition-specific concerns (e.g., *"Stroke Rehabilitation"* or *"Multiple Sclerosis Support"*) rather than requiring alignment with a broader identity category (e.g. *disabled*) or resulting disability (e.g. *blindness*) upfront.

Another flexibility consideration is for support to be designed with more attention to emotional timing and readiness. Several participants described how tools that seemed objectively useful did not feel usable at certain moments because they were still processing loss, uncertainty, anger, or fear. This underscores a need for technologies that clinicians can point to which support different stages of disability experiences without assuming a single emotional trajectory toward acceptance. For example, we can imagine an information synthesis application that responsively presents identity-aligned resources based on the keywords and interests patients search for or based on routine therapist-informed surveys within the application. As patients start their journey, the application may provide resources that are more medical-oriented and based in early health management information. As patients progress, and perhaps start to become curious about lived experience and specific identities, the application could accordingly point to community-oriented sources.

5.2 Designing around labor in new disability navigation

Newly disabled adults repeatedly worked to gather usable information in clinical settings. In many cases, this work was what kept care moving forward.

We interpret one part of these patient practices as a form of **credibility labor**: the work participants undertook to make their symptoms, concerns, and self-knowledge recognizable within constrained and often skeptical clinical systems. This often meant summarizing complex symptom histories between appointments, curating evidence from sources participants believed clinicians would respect, developing potential diagnoses or treatment plans to propose to doctors, and bringing documentation to appointments. Importantly, this labor was shaped by power. In particular, women and non-binary participants often described their symptoms being dismissed as psychosomatic and had to do even more work to be taken seriously, including bringing male partners to appointments and disclosing their academic and professional background to doctors. This finding reflects previous research identifying gaslighting [70] and gender bias in medical care [22, 34, 45, 61]. For newly disabled adults, these dynamics can further delay diagnosis, treatment, accommodations, and access to support, compounding the consequences of dismissal and gaslighting [120, 130, 138]. Our findings build on epistemic injustice scholarship in healthcare [27, 40] by examining the consequences of dismissing lived expertise and highlighting clinical power imbalance during early disability experiences. Specifically, while epistemic injustice describes the credibility deficits patients experience, we identify the concrete forms of labor participants performed to counteract these harms.

We define another part of participant effort as **coordination labor**: the extensive work participants did to manage multiple threads of their healthcare journey in order to ensure ongoing care, at times at the cost of maintaining employment. This included researching symptoms and treatments, coordinating specialists, tracking test results, comparing online sources, interpreting medical records, documenting symptom changes, preparing for appointments, and more. Coordination labor builds on the concept of illness work while illustrating how during early disability experiences, the amount of illness work increasingly disrupts everyday life work [24]. Participants described coordinating healthcare as so consuming that employment, finances, and household management became secondary, with some leaving their jobs to manage their disability full time. This level of work was non-negotiable for participants, although at varying levels, raising questions around how community, socioeconomic, medical access, geographic, and demographic factors could impact the severity of labor in this process [71, 97, 143, 145].

We suggest that credibility and coordination labor fall under the umbrella of invisible labor of access, a documented form of work from the accessibility and HCI research communities. Previous research in this area suggests that the additional labor people with disabilities must perform to create access often goes unnoticed in the contexts they build access within, particularly in mixed-ability settings, such as employment and educational environments [10, 28, 122, 128, 137]. We extend this research by identifying manifestations of invisible labor of access that newly disabled participants had to take on while managing their care in medical environments and in highly collaborative processes with clinicians.

5.2.1 Design Implications. Designing for new disability requires designing for the work people are already doing. Our findings advocate supporting the coordination and credibility labor participants engage in, while considering how information can be made useful across unequal and time-constrained care encounters.

One design direction is to partially shift labor from patients to technology, with appropriate human oversight. Systems should not further reward only those who can already produce polished medical summaries or present as more credible. Designing around these assumptions risks reproducing the same inequities participants were already encountering in care, such as clinical assumptions around gender, and other inequities found in related research, such as language-based discrimination [21, 43, 127]. Instead, we propose tools that draw on clinician-vetted resources to aggregate disability and medical information, reducing the time patients spend searching across the web and piecing together credible information. Tools that better support longitudinal tracking and information aggregation could help people distill large amounts of lived and medical information into forms that are meaningful both to themselves and to providers, therefore reducing the credibility and coordination labor placed on patients.

For these information-seeking components of early disability navigation (e.g., understanding initial symptoms, learning about the healthcare system, exploring potential diagnoses), and the shifting of labor we propose, AI is appealing [13, 93, 147]. Large language model (LLM)-based systems can synthesize research, reduce cognitive load, and deliver information aligned with patients' mental models. Such systems could also serve as a shared reference point during visits, allowing patients and clinicians to review synthesized information together and interrogate potential inaccuracies. Patients could have immediate access to clinician-supported information through interacting with an LLM as an intermediary while waiting for formal, in-person or virtual face time with a doctor.

Rather than encouraging autonomous diagnosis, we envision these systems as supporting clinical communication by helping patients prepare questions, summarize their guiding research, and organize their health history. This orientation positions AI as a scaffold for patient-clinician interaction, not as a replacement for clinical expertise nor as an encouragement for inappropriate self-diagnosis.

However, participants expressed privacy and accuracy concerns when sharing disability-related information with AI systems. Existing scholarship also suggests that LLMs are not currently able to comply with HIPAA[4] in meaningful ways, and HIPAA itself does not yet fully address AI-era privacy concerns [79, 83, 105, 110]. At the same time, recent HCI work demonstrates that people often knowingly disclose sensitive information to AI systems because the perceived benefits (immediate access to information, emotional support, and personalized assistance) outweigh privacy concerns [62, 69].

Accuracy, credibility, and privacy already underlie tensions between patients and clinicians and further complicate AI integration. Future work must explore the potential of AI as a mediator of clinical relationships through its ability to streamline patient research and labor processes. This research must go beyond clinician- and patient-in-the-loop co-design processes to identify vetted sources, reconcile lived experience with formal medical research, and iteratively validate outputs: It must also investigate the substantial work required to guide dataset construction, annotation, and validation safely. Given the high-risk consequences of hallucination and misinformation in medical contexts [12, 46, 81], ongoing clinician oversight would remain necessary even post-deployment, as user warnings and disclosures may not be strong enough signifiers to protect patients. Enforcing human oversight into the system workflow introduces new forms of labor that must be acknowledged and defined, particularly in a healthcare system that is already strained in resources and imposes immense labor on clinicians.

5.2.2 Societal Considerations. We do not suggest that design alone can solve structural failures in healthcare. Short appointments, delayed referrals, medical gaslighting, insurance constraints, and bias are institutional problems. However, HCI and accessibility research can still intervene meaningfully by designing tools that support patient self-advocacy while making visible and addressing the hidden labor that newly disabled people are already performing. As we design these tools, we must be attentive to ensuring technology is not applied as a band-aid to institutional healthcare problems, which in turn exacerbates infrastructural harm. Furthermore, we must be sensitive to existing patient-clinician relationships within these systems by building tools attuned to interpersonal dynamics that rely on communication, credibility, and trust.

5.3 Integrating lived experience into support infrastructures

Although participants' use of social media could be at odds with credibility labor, these spaces became valuable in meeting needs that formal care could not [35, 74, 129]. Clinical encounters often helped participants pursue diagnosis and treatment, but they rarely offered enough space to address broader questions of daily life, adaptation, validation, identity, or what disability might actually look like in practice.

Participants situated social media as a way to leverage lived experience as supplemental expertise. Online communities, such as Facebook groups and Reddit threads, offered lived experience, practical adaptation strategies, and emotional recognition. For some participants, AI tools such as ChatGPT also helped bridge between these forms of support. Participants were often moving among resources strategically, establishing each as offering something different.

5.3.1 Design Implications. Future patient research and support tools must contend with multiple forms of expertise and credibility, moving beyond simplistic binaries such as formal and informal information. Rather than imagining one ideal support system, a more realistic design goal may be to better support navigation across heterogeneous sources of care and knowledge.

There may be value in tools that help people move between medical information, AI-supported synthesis and streamlining, and lived experience without collapsing the distinction between them. Designing while integrating lived experience through community and public forums also requires attending to moderation, trust, and emotional safety: Participants encountered misinformation, misogyny, and ableism on social media. The value of these spaces did not come from them being inherently safe or accurate, but from the validation and practical knowledge that participants could not find elsewhere.

With these considerations in mind, we can imagine an application that allows users to streamline their formal research while also creating space for community-based forums with other patients and pointing to social media content creators, or even specific videos, that discuss diagnoses, lifestyle adaptations, and practical advice. Rather than replacing existing communities, these future systems could preserve their strengths while making risks more visible and manageable in a smaller scale, context-specific space. For example, the imagined application could distinguish lived experience from medical guidance, provide pathways to trusted clinical resources when appropriate, and establish a more moderated setting to minimize harms that participants found in larger social media platforms.

6 Conclusion

Semi-structured interviews with recently disabled adults shed light on how people use technology to navigate various stages of early disability within the United States healthcare system, from experiencing symptoms to seeking specialists and diagnoses to managing ongoing care. We found that participants moved through fragmented medical systems, digital informational tools, online support communities, and shifting understandings of and identification with disability itself. Participants often had to perform substantial **credibility and coordination labor** to secure care, piece together support across multiple technologies to fill in healthcare gaps, and make decisions about using tools and accommodations while negotiating grief, permanence, and disability identity.

Our findings highlight that new disability is not only a medical or personal transition, but also a sociotechnical one. This work calls for more attention to the fluctuating and early phases of disability

[4]HIPAA refers to the Health Insurance Portability and Accountability Act, which regulates patient privacy and data security.

navigation, and raises considerations for HCI research practices using identity-forward language to recruit participants and inadvertently excluding those who may still benefit from assistive tools. We advocate for a shift in labor from patient to technological support while seeking, synthesizing, and presenting health information. Finally, we ask these tools to balance heterogeneous sources of information in patient research processes, such as vetted clinical sources along with social, community-based forums that provide practical and lived experience.

Designing for new disability experiences means that in addition to supporting access, mediating clinical relationships, and alleviating patient labor, we must also design for uncertainty, evolving needs, and people who may not yet see themselves as aligned with disability-oriented systems.

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A Interview Guide

Semi-structured interview questions

- Demographic Questions
    - How old are you?
    - What gender do you identify as?
    - How would you describe your disability? When did you receive your diagnosis for each of them (if multiple)?
- Disability Acquisition and Diagnosis
    - When did you start to exhibit traits associated with [their disability]?
        - What were some of your initial emotions when these symptoms started to appear?
        - Who did you talk to about it?
    - Did you receive a formal diagnosis for these conditions?
        - What prompted you to seek formal diagnosis?
        - Can you walk me through what the process of seeking and receiving your diagnosis was?
            - What was your reaction to receiving this diagnosis?
            - What tools did you use to learn about specific disabilities or to understand information from doctors? Specific websites? Books? Blogs? Chatbots? [Expand with follow up questions on the tools they mention, how they used it, why they used it, challenges, what the tools did well, etc.]
            - Who was your support system at this time? How did they provide support?
        - If no formal diagnosis was sought, why not?
    - How did the symptoms progress over time?
        - What resources did you use to learn more about your symptoms? Doctors? Friends? Books? Etc.
        - What tools/technologies did you use to learn more about your symptoms? Specific websites? Blogs? Chatbots? [Expand with follow up questions on the tools they mention, how they used it, why they used it, challenges, what the tools did well, etc.]
        - Who was your support system as you were noticing these changes? How did they provide support?
        - What were some emotions that you were feeling as you learned more about symptoms and your disability?
    - How will your disability change over time?
        - What are you doing to prepare for these changes? Either emotionally or practically?
- Identity and Lifestyle Changes
    - Do you consider disability as part of your identity? Do you consider yourself part of the disability community? Why?
    - Are there cultural factors that have influenced how you think about your disability? (Cultural background, religion, etc.)

    - Are there social factors that have influenced how you think about your disability? (Friend group, family, workplace, etc.)
    - Can you describe a few lifestyle changes that you have made due to your disability?
        - What tools/technologies are helping you make these lifestyle changes?
        - Who is supporting you through these changes? How do they provide support?
    - Can you describe some of your health management strategies?
    - What resources are you using to continue understanding and adapting to your disability? (Books, movies, social media, music, etc.)
    - Any other tools/technologies are you using to continue understanding and adapting to your disability?

- Have you tried out any tools or apps for your disability that did not work for you? Which ones? What were your challenges with them?
- Are there any tools you wish you had but don't have access to because of certain constraints?
- If you had a magic wand and could instantly create a tool that would help you in any aspect regarding your disability, what would it do?
    - Follow up questions such as: Why that feature/tool format (app, website, etc.)?
    - Who else do you think could benefit from this tool?
- Other Tech Use?
    - We have talked about how [insert tech they have mentioned] has helped you so far. More broadly, are there any other technologies that you use regularly?
- Is there anything else you would like to share that we haven't already covered today?